

Patient Admission Policy

Hospital policy for safe, equitable, and documented inpatient, observation, and procedural admission

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Dataset status

This document is a realistic synthetic artifact created to test document ingestion, chunking, retrieval, metadata filtering, reranking, citation, and question-answering workflows. It has not been reviewed or approved for patient care.

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Controlled use

Printed copies are uncontrolled. Users should verify the effective version, approved local order sets, current formulary information, and governing law before applying any content.

1. Purpose

This policy establishes a reliable admission process that confirms clinical authority, patient identity, care setting, legal status, required notices, and immediate safety needs without delaying emergency stabilization. This controlled section defines the minimum hospital-wide expectation for patient admission and the evidence required to show that the expectation was met. [1-3]

Key considerations are: The process is designed to prevent wrong-patient registration, inappropriate level-of-care assignment, missed isolation or safeguarding needs, duplicate records, and avoidable financial or consent confusion. The requirement applies at every relevant care transition and should be completed using the approved electronic workflow rather than an informal parallel process. Admission begins when an authorized clinician places or communicates an admission decision and ends when the patient is registered, assigned to an accepting service and location, and safely handed over. When information is incomplete or conflicting, staff pause the affected step, verify the source, and document how the discrepancy was resolved. The policy supports

inpatient, observation, same-day procedure, direct, emergency, elective, and transfer admissions while recognizing that each pathway has additional requirements. The responsible role records the decision, supporting information, communication recipient, and any follow-up needed to close the loop.

Within the SVMH care pathway, Use the designated admission pathway and record the admitting clinician, service, status, diagnosis or reason, and destination. The accountable department maintains the approved form, order set, checklist, and escalation contacts and retires superseded versions. Patient Access and Nursing Administration jointly own reliability of the end-to-end admission process. Leaders review compliance data and event reports without discouraging good-faith reporting of near misses or system weaknesses.

2. Scope

The policy applies to all hospital departments and workforce members who request, authorize, register, accept, transport, place, or financially counsel a patient for a hospital encounter. This controlled section defines the minimum hospital-wide expectation for patient admission and the evidence required to show that the expectation was met.

Assessment should address the following: It covers adults and children, voluntary and involuntary pathways where permitted, patients arriving from home or another facility, and patients moving from an outpatient or emergency encounter to a hospital bed. When information is incomplete or conflicting, staff pause the affected step, verify the source, and document how the discrepancy was resolved. Dedicated clinical protocols govern trauma activation, obstetric admission, psychiatric emergency, disaster surge, neonatal care, transplantation, and other specialty pathways. The responsible role records the decision, supporting information, communication recipient, and any follow-up needed to close the loop. Emergency screening and stabilizing treatment are not conditioned on insurance information, advance payment, identification documents, or completion of routine forms. Implementation must be equitable and accessible, including interpreter services, disability accommodation, and a lawful process for personal representatives or support persons.

For routine implementation, Apply the emergency pathway whenever delay for routine admission tasks could worsen an emergency medical condition. Direct verbal communication is required for time-sensitive exceptions; passive inbox messages alone do not constitute closed-loop notification. Department leaders map local workflows to this policy and identify any specialty addendum that supersedes a general step. Education is provided at onboarding, after material revision, and when monitoring shows a knowledge or process gap.

Warning

Administrative or payment questions must never delay emergency medical screening, stabilization, or a medically necessary transfer.

3. Definitions

Standard definitions reduce errors when terms such as inpatient, observation, registration, encounter, accepting service, and responsible practitioner are used across clinical and administrative teams. This controlled section defines the minimum hospital-wide expectation for patient admission and the evidence required to show that the expectation was met.

The practical interpretation is as follows: Admission status is the authorized classification of the encounter and affects clinical workflow, payer notification, notices, and billing but does not determine the patient's right to emergency care. The responsible role records the decision, supporting information, communication recipient, and any follow-up needed to close the loop. The accepting service is the clinical team that has agreed to assume responsibility and has a qualified practitioner available to enter or confirm orders and perform required evaluation. Implementation must be equitable and accessible, including interpreter services, disability accommodation, and a lawful process for personal representatives or support persons. A duplicate medical record is a second enterprise identity created for the same person, while an overlay occurs when information for different people is combined under one record. Urgent clinical needs take priority over routine administrative sequencing, but omitted requirements are completed as soon as the patient is stable.

During assessment and treatment, Use approved terminology in orders, bed requests, handoffs, notices, and financial communication. Supervisors ensure staffing and access to interpreters, identity tools, consent resources, and after-hours decision support needed to perform the workflow. Health Information Management maintains enterprise definitions and the process for merging or separating records. Corrective actions address workflow design, technology, workload, and communication in addition to individual performance when relevant.

Table 1. Admission status and workflow distinctions

Term	Operational meaning	Required safeguard
Inpatient	Formal inpatient admission under an authorized order and applicable criteria	Status order, notices, utilization review, accepting service
Observation	Outpatient monitoring and treatment while disposition is clarified	Correct status, required notice, reassessment and conversion process
Direct admission	Planned arrival accepted without initial emergency registration	Clinical acceptance, bed/acuity confirmation, arrival contingency
Procedural admission	Encounter tied to scheduled intervention or recovery need	Procedure consent, preassessment, medication and fasting review

4. Policy Principles

Admission is patient-centered, nondiscriminatory, clinically led, and proportionate to risk. Care setting decisions are based on clinical need, capability, capacity, and applicable legal or payer requirements. This controlled section defines the minimum hospital-wide expectation for patient admission and the evidence required to show that the expectation was met.

Relevant clinical features include: Patients receive respectful communication in a language and format they can understand and are not denied equal access because of disability, language, race, ethnicity, religion, sex, gender identity, sexual orientation, or ability to pay. Implementation must be equitable and accessible, including interpreter services, disability accommodation, and a lawful process for personal representatives or support persons. The hospital uses the minimum necessary administrative burden while obtaining information needed for identity, treatment, communication, safety, legal compliance, and accurate billing. Urgent clinical needs take priority over routine administrative sequencing, but omitted requirements are completed as soon as the patient is stable. Patient preference is considered in room placement, support-person participation, privacy, cultural practice, and communication unless safety, infection control, or capacity requires a limitation. Staff use the least restrictive and least burdensome method that safely achieves the policy objective and preserves dignity and patient rights.

At each transition of care, Explain material restrictions or delays and document the alternative offered whenever the preferred arrangement cannot be provided. The health record must allow an independent reviewer to reconstruct what occurred, why the decision was made, and who accepted follow-up responsibility. Patient Relations and Compliance review concerns about discriminatory access or inconsistent application of admission requirements. Questions about legal interpretation are referred to Privacy, Compliance, Risk Management, or Legal Counsel as appropriate.

5. Roles and Responsibilities

Admission requires coordinated action by the admitting practitioner, nursing, Patient Access, bed management, utilization management, finance, transport, pharmacy, and ancillary services. This controlled section defines the minimum hospital-wide expectation for patient admission and the evidence required to show that the expectation was met.

The core elements are: The admitting practitioner confirms the need, status, service, preliminary diagnosis, immediate orders, required monitoring, and any isolation or special capability. Urgent clinical needs take priority over routine administrative sequencing, but omitted requirements are completed as soon as the patient is stable. Patient Access verifies identity and demographics, creates or locates the correct encounter, obtains signatures and notices when appropriate, and avoids duplicate-record creation. Staff use the least restrictive and least burdensome method that safely achieves the policy objective and preserves dignity and patient rights. Nursing

performs arrival assessment, verifies orders and allergies, initiates safety precautions, secures belongings and medicines, and escalates any mismatch between requested and actual acuity. The process should identify a named owner and deadline for unresolved items so they do not disappear at handoff, transfer, or discharge.

When the guideline is applied in practice, Bed management assigns a location only after confirming clinical suitability, staffing, isolation, equipment, and acceptance by the receiving unit. The accountable department maintains the approved form, order set, checklist, and escalation contacts and retires superseded versions. Each role maintains competency in the admission tasks assigned to it and understands when to escalate rather than proceed. Leaders review compliance data and event reports without discouraging good-faith reporting of near misses or system weaknesses.

Table 2. Core responsibility matrix

Role	Primary admission responsibility	Escalation example
Admitting practitioner	Clinical decision, status, service, orders, medical assessment	Uncertain level of care or unavailable specialty
Patient Access	Identity, demographics, consent/notice workflow, encounter integrity	Possible duplicate, identity mismatch, legal representative issue
Receiving nurse	Arrival assessment, identification, safety, medication and belongings review	Acuity exceeds unit capability or orders missing
Bed management	Capacity, placement, isolation, equipment and acceptance	No suitable bed or conflicting priorities

6. Admission Authorization and Clinical Acceptance

Every nonemergency admission requires an authorized order or equivalent documented instruction and acceptance by a service capable of managing the patient at the planned level of care. This controlled section defines the minimum hospital-wide expectation for patient admission and the evidence required to show that the expectation was met.

Key considerations are: The request includes patient identity, diagnosis or reason, current condition, required monitoring, isolation status, mobility and behavioral risks, special equipment, and expected arrival time. Staff use the least restrictive and least burdensome method that safely achieves the policy objective and preserves dignity and patient rights. Direct admissions are reassessed on arrival because condition and bed availability may change between acceptance and presentation. The process should identify a named owner and deadline for unresolved items so they do not disappear at handoff, transfer, or discharge. When the patient is clinically unstable or the requested unit cannot safely meet needs, the patient is redirected to the emergency or higher-acuity pathway without waiting for routine placement. Any suspected safety, privacy, legal, billing, or identity issue is escalated through the designated department rather than managed through an undocumented workaround.

Within the SVMH care pathway, The accepting practitioner and receiving unit confirm acceptance before transport from another facility or nonemergency arrival whenever feasible. Direct verbal communication is required for time-sensitive exceptions; passive inbox messages alone do not constitute closed-loop notification. Medical Staff Services defines which practitioner categories may admit and which require supervising or attending confirmation. Education is provided at onboarding, after material revision, and when monitoring shows a knowledge or process gap.

7. Identity Verification and Record Integrity

Accurate patient identification is required before orders, specimens, medicines, procedures, wristbands, or health information are associated with the encounter. This controlled section defines the minimum hospital-wide expectation for patient admission and the evidence required to show that the expectation was met. [4]

Assessment should address the following: Use at least two approved identifiers and compare reliable source information with the enterprise record; room number or physical location is not an identifier. The process should identify a named owner and deadline for unresolved items so they do not disappear at handoff, transfer, or discharge. For unidentified, unconscious, newborn, trauma, or emergency patients, create a temporary identity

using the approved convention and reconcile it as soon as reliable information becomes available. Any suspected safety, privacy, legal, billing, or identity issue is escalated through the designated department rather than managed through an undocumented workaround. Potential duplicates, identity theft, overlays, alias use, confidential encounters, and demographic conflicts are referred to the designated identity-integrity team. The requirement applies at every relevant care transition and should be completed using the approved electronic workflow rather than an informal parallel process.

For routine implementation, Print and apply identification bands only after the encounter and identifiers are verified, then have clinical staff recheck them at handoff. Supervisors ensure staffing and access to interpreters, identity tools, consent resources, and after-hours decision support needed to perform the workflow. Health Information Management audits duplicate rates, overlays, merges, demographic corrections, and wrong-patient near misses. Corrective actions address workflow design, technology, workload, and communication in addition to individual performance when relevant.

Warning

Do not merge records, copy clinical information between records, or relabel specimens to correct an identity concern without following the approved identity-integrity process.

8. Registration, Consent, and Required Notices

Registration collects information needed to identify the patient, communicate, coordinate care, meet legal obligations, and submit accurate claims. Consent and notices are obtained according to capacity, urgency, and applicable law. This controlled section defines the minimum hospital-wide expectation for patient admission and the evidence required to show that the expectation was met.

The practical interpretation is as follows: Staff identify the patient or authorized representative, explain the form or notice, provide interpreter or accessibility support, and record refusal or inability without misrepresenting a signature. Any suspected safety, privacy, legal, billing, or identity issue is escalated through the designated department rather than managed through an undocumented workaround. General consent does not replace procedure-specific informed consent, research consent, authorization for a nonroutine disclosure, or other specialized requirements. The requirement applies at every relevant care transition and should be completed using the approved electronic workflow rather than an informal parallel process. Insurance and financial information are collected after immediate clinical needs are addressed and are verified without promising coverage or final payment amount. When information is incomplete or conflicting, staff pause the affected step, verify the source, and document how the discrepancy was resolved.

During assessment and treatment, Provide copies of signed forms or instructions for obtaining them and document the method when electronic or remote consent is used. The health record must allow an independent reviewer to reconstruct what occurred, why the decision was made, and who accepted follow-up responsibility. Legal Counsel, Privacy, and Patient Access approve standard consent language and required notices. Questions about legal interpretation are referred to Privacy, Compliance, Risk Management, or Legal Counsel as appropriate.

9. Initial Clinical and Safety Assessment

The receiving clinical team completes an assessment appropriate to the patient's condition and the unit, identifying immediate treatment needs and risks that require precautions or consultation. This controlled section defines the minimum hospital-wide expectation for patient admission and the evidence required to show that the expectation was met.

Relevant clinical features include: Assessment includes vital signs, pain, allergies, medicines, suicide or violence risk when indicated, falls, skin, nutrition, mobility, infection precautions, substance use, safeguarding, and communication needs. The requirement applies at every relevant care transition and should be completed using the approved electronic workflow rather than an informal parallel process. Medication reconciliation identifies what the patient was actually taking, the last dose, nonprescription products, high-alert medicines, and who can verify uncertain information. When information is incomplete or conflicting, staff pause the affected step, verify

the source, and document how the discrepancy was resolved. Belongings, valuables, home medicines, devices, weapons, hazardous items, and assistive equipment are managed under the applicable policy. The responsible role records the decision, supporting information, communication recipient, and any follow-up needed to close the loop.

At each transition of care, Initiate precautions and urgent treatment immediately and complete nonurgent assessment elements within the time frame assigned by the receiving unit standard. The accountable department maintains the approved form, order set, checklist, and escalation contacts and retires superseded versions. Nursing leadership monitors completion and quality of admission assessment rather than signature alone. Leaders review compliance data and event reports without discouraging good-faith reporting of near misses or system weaknesses.

Table 3. Examples of high-priority admission findings

Finding	Immediate action	Required communication
Unstable vital signs or deterioration	Stabilize and activate escalation pathway	Responsible practitioner and higher-acuity team
Possible infectious isolation need	Apply indicated precautions pending evaluation	Infection Prevention and receiving staff
Suicide, violence, or elopement risk	Use approved safety precautions and environment	Practitioner, nursing leader, behavioral-health team
Medication uncertainty	Hold unsafe assumptions; verify sources	Pharmacist and prescribing clinician

10. Bed Placement, Capacity, and Internal Handoff

Placement must match clinical acuity, monitoring, isolation, staffing, equipment, age, behavioral needs, and specialty capability. Capacity pressure does not justify unsafe placement. This controlled section defines the minimum hospital-wide expectation for patient admission and the evidence required to show that the expectation was met.

The core elements are: Cohorting and room assignment follow infection-prevention, privacy, safety, and patient-rights requirements. When information is incomplete or conflicting, staff pause the affected step, verify the source, and document how the discrepancy was resolved. The sending and receiving teams exchange diagnosis, current condition, treatment given, pending tests, allergies, medicines, precautions, code status, and anticipated problems. The responsible role records the decision, supporting information, communication recipient, and any follow-up needed to close the loop. A patient who arrives on a unit with needs beyond capability remains under active clinical supervision while transfer to an appropriate location is arranged. Implementation must be equitable and accessible, including interpreter services, disability accommodation, and a lawful process for personal representatives or support persons.

When the guideline is applied in practice, Use the standardized handoff and obtain receiving acknowledgment before responsibility is considered transferred. Direct verbal communication is required for time-sensitive exceptions; passive inbox messages alone do not constitute closed-loop notification. The house supervisor or command structure resolves competing placement priorities and records significant capacity-related delays. Education is provided at onboarding, after material revision, and when monitoring shows a knowledge or process gap.

11. Special Populations and Legal Status

Additional safeguards apply when a patient is a minor, lacks decision-making capacity, is in custody, requires confidential status, has a disability, or is admitted under a behavioral-health or public-health authority. This controlled section defines the minimum hospital-wide expectation for patient admission and the evidence required to show that the expectation was met.

Key considerations are: Staff verify the authority of a parent, guardian, health-care agent, or other representative and distinguish decision-making authority from permission to receive information or visit. The responsible role records the decision, supporting information, communication recipient, and any follow-up needed to close the loop. Patients in law-enforcement custody receive clinically necessary care and privacy to the extent possible,

with restraints and officer presence managed according to safety and law. Implementation must be equitable and accessible, including interpreter services, disability accommodation, and a lawful process for personal representatives or support persons. Confidential or restricted encounters use approved directory, communication, and access controls without creating unsafe barriers to clinical information. Urgent clinical needs take priority over routine administrative sequencing, but omitted requirements are completed as soon as the patient is stable.

Within the SVMH care pathway, Consult Risk Management, Legal Counsel, Ethics, Social Work, or Security when authority, custody, abuse, trafficking, or confidentiality is unclear. Supervisors ensure staffing and access to interpreters, identity tools, consent resources, and after-hours decision support needed to perform the workflow. Policies for minors, behavioral health, safeguarding, disability accommodation, and custody remain controlling for their specialized requirements. Corrective actions address workflow design, technology, workload, and communication in addition to individual performance when relevant.

12. Financial Communication and Coverage Verification

Financial counseling is transparent, respectful, and separate from emergency clinical decision-making. Estimates and coverage information are clearly described as estimates rather than guarantees. This controlled section defines the minimum hospital-wide expectation for patient admission and the evidence required to show that the expectation was met. [2,5]

Assessment should address the following: Patient Access obtains insurance details, seeks authorization when required, explains known network or benefit concerns, and refers questions to trained financial counselors. Implementation must be equitable and accessible, including interpreter services, disability accommodation, and a lawful process for personal representatives or support persons. The patient is informed about available estimates, price-transparency resources, financial-assistance screening, payment methods, and whom to contact about a bill. Urgent clinical needs take priority over routine administrative sequencing, but omitted requirements are completed as soon as the patient is stable. A patient who cannot pay a requested deposit receives the same emergency evaluation and is offered the hospital's assistance and payment processes for scheduled services. Staff use the least restrictive and least burdensome method that safely achieves the policy objective and preserves dignity and patient rights.

For routine implementation, Document material financial communication and any decision to proceed, postpone, or refer a nonemergency service for review. The health record must allow an independent reviewer to reconstruct what occurred, why the decision was made, and who accepted follow-up responsibility. Revenue Cycle leadership monitors whether financial processes are applied consistently and without discriminatory impact. Questions about legal interpretation are referred to Privacy, Compliance, Risk Management, or Legal Counsel as appropriate.

13. Documentation and Communication Standards

The admission record must support safe care, correct patient identity, regulatory review, utilization management, billing, and continuity across disciplines. This controlled section defines the minimum hospital-wide expectation for patient admission and the evidence required to show that the expectation was met.

The practical interpretation is as follows: Required elements include arrival and admission times, source, mode, status, service, responsible practitioner, diagnosis or reason, identity verification, consent or notice status, and location. Urgent clinical needs take priority over routine administrative sequencing, but omitted requirements are completed as soon as the patient is stable. Clinical documentation includes initial assessment, medication reconciliation, allergies, precautions, orders, belongings, education, support person, and unresolved items. Staff use the least restrictive and least burdensome method that safely achieves the policy objective and preserves dignity and patient rights. Corrections follow the health-record amendment process and never conceal the original entry or attribute work to a person who did not perform it. The process should identify a named owner and deadline for unresolved items so they do not disappear at handoff, transfer, or discharge.

During assessment and treatment, Use structured fields for searchable data and narrative when context, uncertainty, or decision rationale cannot be represented safely in a checkbox. The accountable department

maintains the approved form, order set, checklist, and escalation contacts and retires superseded versions. Health Information Management and Quality review documentation completeness and integrity using risk-based sampling. Leaders review compliance data and event reports without discouraging good-faith reporting of near misses or system weaknesses.

14. Compliance, Monitoring, and Corrective Action

Compliance is assessed through workflow metrics, record review, patient feedback, identity events, placement delays, missed notices, and adverse-event analysis. This controlled section defines the minimum hospital-wide expectation for patient admission and the evidence required to show that the expectation was met.

Relevant clinical features include: Metrics may include duplicate record creation, admission-order completeness, medication-reconciliation completion, time to appropriate bed, interpreter use, notice delivery, and unresolved handoff items. Staff use the least restrictive and least burdensome method that safely achieves the policy objective and preserves dignity and patient rights. Serious wrong-patient, unsafe placement, discrimination, consent, privacy, or emergency-access concerns are reported through the safety or compliance system. The process should identify a named owner and deadline for unresolved items so they do not disappear at handoff, transfer, or discharge. Audits distinguish a documentation omission from a process that did not occur and verify corrective action through repeat measurement. Any suspected safety, privacy, legal, billing, or identity issue is escalated through the designated department rather than managed through an undocumented workaround.

At each transition of care, Policy owners review trends at least quarterly and assign improvement actions with an accountable leader and completion date. Direct verbal communication is required for time-sensitive exceptions; passive inbox messages alone do not constitute closed-loop notification. Persistent or material noncompliance is escalated to the appropriate executive, medical staff, compliance, privacy, or quality committee. Education is provided at onboarding, after material revision, and when monitoring shows a knowledge or process gap.

Table 4. Example admission quality indicators

Indicator	Review question	Primary owner
Identity integrity	Were two identifiers verified and duplicate risk addressed?	Patient Access / HIM
Clinical readiness	Were orders, assessment, precautions, and handoff complete?	Medical / Nursing
Placement safety	Did location match acuity, isolation, staffing, and equipment?	Bed Management
Equitable access	Were emergency care and accommodation provided without improper barrier?	Compliance / Patient Relations

15. Exceptions, Downtime, and Policy Review

Exceptions are limited to emergency necessity, system downtime, disaster operations, legal requirements, or a documented patient-specific circumstance that makes the standard step unsafe or impossible. This controlled section defines the minimum hospital-wide expectation for patient admission and the evidence required to show that the expectation was met.

The core elements are: During electronic downtime, use approved paper identifiers, forms, labels, orders, and logs and reconcile them to the correct electronic encounter when systems recover. The process should identify a named owner and deadline for unresolved items so they do not disappear at handoff, transfer, or discharge. A deviation identifies the reason, authorizing role, temporary safeguard, communication performed, and time or condition for returning to the standard process. Any suspected safety, privacy, legal, billing, or identity issue is escalated through the designated department rather than managed through an undocumented workaround. Routine workload, preference, unfamiliarity, or lack of planning is not an acceptable standing exception. The requirement applies at every relevant care transition and should be completed using the approved electronic workflow rather than an informal parallel process.

When the guideline is applied in practice, Report repeated exceptions so that leaders can correct capacity, technology, training, or workflow problems. Supervisors ensure staffing and access to interpreters, identity tools, consent resources, and after-hours decision support needed to perform the workflow. The policy owner reviews

the document at least every two years and after major regulatory, accreditation, technology, or safety change. Corrective actions address workflow design, technology, workload, and communication in addition to individual performance when relevant.

Practice note

Printed copies are uncontrolled; verify the current approved version and linked procedures before use.

16. References

References are provided to make the synthetic document realistic and to support citation and provenance tests. They should be checked against the most current source before any real-world use.

[1] Centers for Medicare & Medicaid Services. Conditions of Participation for Hospitals, including patient rights, medical record, utilization review, and discharge planning requirements. Current edition.

[2] Centers for Medicare & Medicaid Services. Emergency Medical Treatment and Labor Act guidance and interpretive resources. Current edition.

[3] Centers for Medicare & Medicaid Services. Medicare Claims Processing Manual, Chapter 2: Admission and Registration Requirements. Current edition.

[4] The Joint Commission. National Patient Safety Goals: use at least two patient identifiers. Current hospital program edition.

[5] Centers for Medicare & Medicaid Services. Hospital Price Transparency and No Surprises Act resources. Current edition.

[6] U.S. Department of Health and Human Services, Office for Civil Rights. HIPAA Privacy Rule guidance. Current edition.

Reference status

External guidelines and regulations may change between scheduled reviews. The document owner is responsible for checking high-impact updates and initiating an interim revision when necessary.